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2014).

3.2. The Government's Policy on Supporting Service Users with Long-Term Conditions

Long-term conditions are conditions that currently cannot be cured, but can be managed with medication and other therapies and are a great challenge for the NHS (Department of Health, 2012). Around 15 million people in England have 1 or more long-term conditions. This population represents around 70% of the total cost of NHS and social care (Department of Health, 2012). The magnitude of this challenge has led successive Governments to formulate policy measures to move care towards community and self-management.

The Coalition government continued on from Labour's existing work on personalisation and patient activation through programmes like the Expert Patient Programme. Personal health budgets, introduced from 2009 and piloted more widely from 2014, provided some autonomy to the way in which the care of some service users was set up and delivered (Lymbony, 2012). Ham (2009)

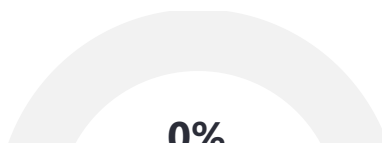
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Task 3: The Impact of Previous and Current Healthcare Policy on Service User Groups

3.1. A Policy Developed During the Coalition Government and Its Impact on Service Users

The most significant policy of the Coalition has been the Health and Social Care Act (2012), which reorganised how the NHS is commissioned and introduced a wide-ranging application of competitive tendering. The impacts to the service users were mixed. There was considerable institutional change when Primary Care Trusts were replaced by CCGs, with Timmins (2012) noting that there was a management uncertainty period when the reorganisation was taking place, during which the relationships between commissioners and providers were broken and there was no clear benefit to patients. The estimated cost of the transition was £3 billion, a significant reallocation of resources from frontline care.

The marketisation aspects of the Act caused much controversy, especially concerning potentially vulnerable service users. Paton (2014) found that the introduction of competitive tendering in community health services resulted in the "fragmentation of care pathways" in some localities, to the detriment of people with complex or multiple needs such as older people and people with learning disabilities. Service users with intellectual disabilities were reported to have lost their continuity of care and access to familiar service workers, due to contract changes as part of the competitive procurement, and this had a clear and negative impact on wellbeing (Paton, 2014).

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which the care of some service users was set-up and delivered (Lymbery, 2012). Ham (2009) saw it as a positive move towards real person-centred care, but warned that the success of personal budgets hinged on the nature of the support planning put in place to support individuals and this was found to be highly variable between localities.

Marmot (2010) stated that long-term conditions do not occur at random in society, but are concentrated in regions of deprivation, with people contracting conditions prematurely and the severity of their conditions worsens in such areas. Policy emphasises on individual behaviour change need to be set against the backdrop of the social determinants of health of the individual, or service-user, for whom health is largely a consequence of structural disadvantage, the whole of which cannot be changed by them (World Health Organization, 2023). To some extent this was recognised in the NHS Long Term Plan (NHS England, 2019) with pledges to "act on social prescribing and prevention," although critics pointed out that these aspirations were undermined by ongoing reductions to local authority public health budgets since 2013.



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